

Level-of-Care Defense Memo

Retrospective observation status downgrade — chest pain admission with troponin elevation and hemodynamic monitoring. Two-Midnight Rule prospective analysis.

Specimen only. Fictional case. No PHI. No real patient, provider, or payer. This memo demonstrates Clinovian's clinical escalation format for denial teams. It is not legal advice, not a treating-physician opinion, and not a binding U.S. medical determination. Proprietary criteria content has been generalized; actual deliverables reference the applicable licensed criteria framework as documented in the case record. Clinical escalation methodology led by an MBBS physician with payer-side utilization-management review experience; clinical reasoning support only, not a licensed clinical opinion.

SECTION 01

Case Snapshot

DENIED SERVICE	Inpatient admission — retrospectively downgraded to observation status
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PLAN TYPE	Commercial PPO
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ADMISSION	42-hour hospitalization (one midnight) for chest pain with troponin elevation, rule-out ACS
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DENIAL BASIS	"Retrospective review indicates observation status was appropriate. Two-midnight benchmark not met. Clinical presentation did not require inpatient-level care."
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CURRENT
LEVEL

Post-discharge retrospective appeal (Level 1)

CLINOVIAN
VERDICT

Strong Appealability

SECTION 02

Appealability Verdict

Strong

The denial applies retrospective knowledge to a prospective clinical-judgment question. At the time of admission, the treating physician had a clinically reasonable expectation that the patient would require inpatient care spanning at least two midnights. The clinical picture at presentation — chest pain with troponin elevation, dynamic ECG changes, history of coronary artery disease with prior stenting, and hemodynamic instability requiring monitoring — supported the admitting physician's judgment. The Two-Midnight Rule evaluates the reasonableness of the physician's expectation at the point of admission, not whether the patient ultimately stayed two midnights in retrospect.

SECTION 03

Payer Denial Rationale Summary

The payer denied inpatient status on three grounds:

Two-midnight benchmark not met: The patient was discharged after 42 hours. The payer argues this is below the two-midnight threshold.

Observation-appropriate presentation: "Chest pain with troponin elevation is commonly managed in an observation setting with serial monitoring."

Retrospective clinical course: "The patient's subsequent workup was negative for acute coronary syndrome, confirming that the presentation did not require inpatient-level intervention."

The denial rationale contains a fundamental analytical error: it uses the outcome of the workup (negative for ACS) to retroactively reclassify the clinical severity at the time of admission. The Two-Midnight Rule explicitly prohibits this retrospective reasoning. The relevant question is what the treating physician reasonably expected at admission — not what the final diagnosis turned out to be.

SECTION 04

Clinical Facts That Matter

Presentation at admission: 64-year-old male presenting to ED with acute substernal chest pain radiating to the left arm, rated 8/10, associated with diaphoresis and nausea. History of coronary artery disease with LAD stent placement (18 months prior). Current on dual antiplatelet therapy.

Initial troponin and ECG: Initial high-sensitivity troponin I: 0.08 ng/mL (above institutional upper reference limit of 0.04). ECG showing ST-segment depression in leads V4–V6 and T-wave inversions in leads I and aVL — representing dynamic changes from prior baseline ECG on file. These findings, in the setting of known coronary disease, constituted an intermediate-to-high-risk presentation for acute coronary syndrome at the time of evaluation.

Hemodynamic monitoring requirement: Blood pressure labile in ED: 168/94 → 92/58 after initial nitroglycerin administration. Cardiology consulted in ED. Continuous telemetry monitoring initiated due to dynamic ECG changes and hemodynamic lability. IV heparin drip initiated per ACS protocol.

Serial troponin trend: Second troponin at 6 hours: 0.06 ng/mL (trending down). Third troponin at 12 hours: 0.04 ng/mL (normalizing). This downward trend ultimately argued against acute MI but was not available to the admitting physician at the time the admission decision was made.

Stress testing and catheterization decision: Pharmacologic stress test performed Day 2 — showed no inducible ischemia. Cardiology determined catheterization not indicated based on stress results. This is the information that supported discharge — and it was available only after the inpatient workup the payer now claims was unnecessary.

Comorbidities affecting risk stratification: Prior LAD stent, type 2 diabetes mellitus, hypertension, hyperlipidemia, chronic kidney disease Stage IIIA (eGFR 48 — relevant to contrast risk in potential catheterization). HEART score at presentation: 7 (high risk).

SECTION 05

Criteria-Style Reasoning — Two-Midnight Rule Analysis

TWO-MIDNIGHT ELEMENT	RECORD EVIDENCE	APPEAL USE
Physician expectation at admission	Admitting physician order documents: "Admit inpatient for serial troponins, continuous	The physician's documented expectation at admission — not the retrospective outcome —

	telemetry, and cardiology evaluation. Anticipate stress test and possible catheterization — expected stay 2+ days."	is the Two-Midnight standard. The expectation of 2+ days was clinically reasonable given the presentation.
Clinical basis for expectation	Troponin elevation, dynamic ECG changes, hemodynamic lability, known CAD with prior stent, HEART score 7 (high-risk), planned serial monitoring + stress testing + potential catheterization	Each of these elements independently supports a reasonable expectation of a multi-day inpatient stay. Collectively, they make the admission decision unambiguously reasonable at the time it was made.
Intervening factor explaining shorter stay	Troponin trended down faster than anticipated. Stress test was negative. Catheterization not needed. Patient stabilized and was safely dischargeable at 42 hours.	A good clinical outcome does not retroactively invalidate the admission. The patient's favorable response to monitoring and workup is the <i>reason</i> the stay was shorter — not evidence that the stay was unnecessary.

SECTION 06

Where the Denial Logic Is Weak

PAYER ASSERTION

"The patient was discharged in 42 hours, below the two-midnight benchmark."

RESPONSE

The Two-Midnight Rule is not a length-of-stay minimum. It is a prospective-judgment standard. The payer's own denial invoked this benchmark, and applied correctly, it evaluates the physician's expectation at the time of admission — not the actual length of stay. A patient who recovers faster than expected does not retroactively become an observation patient.

PAYER ASSERTION

"Chest pain with troponin elevation is commonly managed in observation."

RESPONSE

This generalizes across a heterogeneous population. This patient had prior CAD with stenting, dynamic ECG changes, hemodynamic instability, HEART score 7, CKD affecting contrast decisions, and required IV heparin anticoagulation. Observation-level care was clinically inappropriate for this specific risk profile at the time of presentation.

PAYER ASSERTION

"Final diagnosis was not ACS, confirming observation was appropriate."

RESPONSE

Using the final diagnosis to retroactively determine the appropriate admission status contradicts the prospective-judgment standard the payer itself invoked. The standard is what the physician reasonably expected at admission. Ruling out ACS is the purpose of the inpatient workup — not evidence against it.

Documentation Gaps

Gap 1 — Admission order specificity: The admitting physician's order documents "anticipated stay 2+ days" but does not list the specific clinical factors supporting this expectation in the order itself. The evidence exists elsewhere in the record (ED notes, cardiology consult, nursing assessment), but a single admission-order note linking the clinical presentation to the two-midnight expectation would strengthen the appeal.

Gap 2 — Cardiology consult timing: Cardiology was consulted in the ED and saw the patient within 2 hours, but the consult note was finalized 8 hours after admission. The appeal should reference the consultation timestamp, not the note-finalization timestamp, to demonstrate that specialist evaluation occurred prior to the admission decision.

Recommendation: Attach a short physician attestation to the appeal confirming the clinical reasoning at the time of admission. This is a documentation supplement, not a retrospective justification. The clinical facts already support the admission; the attestation connects them explicitly to the Two-Midnight standard.

Appeal-Writer Instructions

Lead with the Two-Midnight standard, not the clinical narrative. The appeal should open by establishing the applicable standard: inpatient status is evaluated based on the physician's reasonable expectation at the time of admission — the same prospective-judgment benchmark the payer cited in its denial. Then map the clinical evidence to that standard.

Do not concede the discharge timing. The 42-hour stay is not a weakness — it is evidence of effective inpatient management. Frame it as: "The patient's clinical improvement within 42 hours reflects the effectiveness of the inpatient-level monitoring and intervention that the admission provided."

Include the HEART score. A documented HEART score of 7 places this patient in the high-risk category. ACC/AHA guidelines recommend inpatient evaluation for high-risk chest pain presentations. This is an evidence-based clinical standard, not an opinion.

Attach: (1) ED physician note with presentation details, (2) initial ECG and comparison to prior baseline, (3) serial troponin values with timestamps, (4) cardiology consult, (5) admitting physician attestation (if supplemented), (6) stress test results showing the workup that required inpatient setting.

SECTION 09

Risk-of-Failure Assessment

FACTOR	ASSESSMENT
Regulatory alignment	Strong. The prospective-judgment standard the payer invoked supports the admission decision; CMS Two-Midnight Rule guidance (binding for Medicare, widely adopted by commercial payers) confirms this framework.
Clinical evidence strength	Strong. Troponin elevation, dynamic ECG changes, hemodynamic lability, and risk-score documentation are objective and well-documented.

Documentation quality

Moderate. Admission order could be more explicit about two-midnight expectation. Physician attestation supplement recommended.

Payer precedent

Moderate risk. Retrospective observation downgrades on chest pain admissions are a systematic payer strategy. Reviewers are experienced at this category. The appeal must be precise, not general.

Overall: Strong case. With the documentation supplement, this case should prevail at Level 1 or Level 2 appeal. Without it, the case is still moderate-to-strong but relies on the reviewer reading across multiple chart sections to assemble the two-midnight argument.

OPERATIONAL NOTE**Prevention Note**

For future similar cases: This denial was preventable with a single documentation change. If the admitting physician had included a brief attestation in the admission order explicitly linking the clinical presentation (troponin elevation, dynamic ECG changes, hemodynamic lability) to the expectation of an inpatient stay spanning two or more midnights, the Two-Midnight standard would have been documented at the point of admission. Many institutions have implemented a short admission-attestation template for this purpose. The clinical judgment was correct — only the documentation of that judgment was incomplete.

Forward-looking operational insight for the client's physician and CDI teams.

SECTION 10

Internal Pattern Tags

LOC: OBSERVATION DOWNGRADE

TWO-MIDNIGHT RULE

CARDIAC: CHEST PAIN / ACS RULE-OUT

RETROSPECTIVE RECLASSIFICATION

PAYER: COMMERCIAL PPO

APPEALABILITY: STRONG

Want this level of reasoning on your own cases?

START A 3-CASE EVALUATION

SEND A DE-IDENTIFIED CASE

CASE SUITABILITY

Clinovian

This specimen demonstrates the Observation vs. Inpatient Defense Memo — the physician-level Two-Midnight Rule analysis that answers the payer's retrospective reclassification with prospective clinical-judgment evidence. Observation downgrades are one of the most common and most consistently under-argued denial categories.

[Start a 3-case evaluation →](#) · [Send a de-identified case →](#)

